



This is not about  
pathologizing faith.

# A Note on Framing

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This work focuses on harmful sexual messaging, not religion itself.

Faith can be a source of meaning, identity, ethics, and belonging.

Clinical concern arises when sexual teachings become fear-based, shame-based, coercive, or rigid.

The goal is not to remove values.

The goal is to help clients distinguish chosen values from inherited fear.



What did you learn  
about sex before you  
had language for it?

# Early Sexual Messages

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Childhood sexual messages are often absorbed before they are questioned. Explore:

What was said directly?

What was implied?

Who taught you what was "pure," "dangerous," or "wrong"?

What messages still live in your body or relationship?

*Clinical note: The earliest sexual scripts often become automatic adult reactions.*

THE SHAME WE CARRY

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Guilt says,  
"I did something wrong."

Shame says,  
"I am wrong."

# Guilt vs. Shame

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Guilt and shame may look similar but function differently.

Guilt can be connected to behavior or values.

Shame attaches to identity and worth.

Sexual shame may create avoidance, shutdown, secrecy, or distress.

Therapy can help clients move from self-condemnation toward integration.



The body may remember  
what the mind  
has outgrown.

# Embodied Responses

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Clients may cognitively reject rigid sexual teachings but still experience embodied responses.  
Examples:

Tension or pain

Dissociation

Low desire

Anxiety during intimacy

Difficulty receiving pleasure

Clinical note: Do not rush insight into behavior change. Safety often has to be rebuilt through the body.

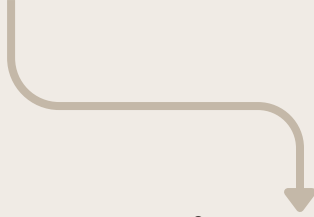




The pathway from  
teaching to distress  
is rarely linear.

# A Clinical Map

**Childhood sexual teachings**



**Internalized sexual scripts**



**Shame, guilt, fear, or confusion**



**Clinical repair, agency, and relational safety**



**Avoidance, low desire, pain, dissociation, or relational conflict**



How it shows up  
in the bedroom.

# COMMON PRESENTATIONS

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**Rigid sexual scripts may appear as:**

- Low desire or desire discrepancy
- Sexual avoidance
- Pain or tension
- Guilt after pleasure
- Difficulty initiating
- Dissociation or "checking out"
- Conflict around roles, expectations, or morality

## CLINICAL NOTE:

The presenting issue may be sexual, but the wound may be relational, spiritual, embodied, or systemic.



The couple is not  
the problem.  
The script may be.

# Externalizing the Script

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MFTs can help couples externalize inherited sexual scripts.

An MFT can support:

Naming the script

Reducing blame

Increasing emotional safety

Improving sexual communication

Clarifying shared and individual values

Renegotiating intimacy with consent and care



Clinical repair  
begins with choice.

# Pathways to Repair

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Meaningful care may include:

Family-of-origin exploration

Values clarification

Shame reduction

Emotion regulation

Attachment and safety work

Consent-centered intimacy conversations

Collaboration with sex therapy, pelvic health, trauma, or medical providers

Reflection question:

What would intimacy look like if it were guided by safety, consent, values, and connection rather than fear?